The Ethics of Circumcision

Brian D. Earp

University of Oxford

Author's copy of an accepted book chapter, in press. Please cite as:

Earp, B. D. (2022). The ethics of circumcision. In Ezio Di Nucci, Ji-Young Lee, Isaac Wagner (eds), *The Rowman & Littlefield Handbook of Bioethics*. Lanham, Maryland: Rowman & Littlefield. In Press. Available online ahead of print at https://www.researchgate.net/publication/360554804_The_Ethics_of_Circumcision

Abstract

In many cultures, children with intersex traits are subjected to medically unnecessary genital operations in an attempt to reshape their sexual anatomy to approximate a more stereotypical male or female appearance. In addition to these surgeries, there are three main patterns of medically unnecessary genital operations performed on non-intersex children across the globe. In the first pattern, the dominant culture endorses routine or religious genital cutting only of boys, as in the United States, Israel, Afghanistan, or Saudi Arabia, for instance. In these cultures, girls are ineligible for genital cutting, or may be seen as deserving protection from it. In the second pattern, neither boys nor girls of the dominant culture have their genitals cut unless there is a strict medical indication—which in these contexts is rare. This is the prevailing pattern throughout Europe and indeed most other parts of the world. In the third pattern, both boys and girls of the dominant culture have their genitals cut for non-medical reasons, for example, in the context of a rite of passage or religious initiation. Typically, this occurs around puberty, but the timing ranges from right after birth to before marriage. In this chapter, I present a moral argument in favor of the second, "European" pattern, both as a matter of ethics and gender equality. I do not discuss legal implications. In other words, I argue that, morally speaking, neither boys nor girls who are too young to consent should have their genitals cut—to any extent—unless it is medically necessary to do so, whether or not they have intersex traits. The alternative, from a gender equality perspective, would be to argue that both boys and girls should have their genitals cut on an equal basis, for example, by a doctor using sterile instruments. But from an ethical perspective, I argue, this would not be acceptable.

The Ethics of Circumcision

Why have a chapter on circumcision in a handbook about bioethics? To North American readers, especially those raised in the United States, the topic may seem a puzzling choice: routine infant penile circumcision—that is, partial or total removal of the foreskin of the penis—is a majority birth custom in U.S. culture, occurring over a million times per year.[1] The surgery is mostly carried out by doctors on babies that have been categorized as male at birth (typically due to the presence of a readily identifiable penis) before they leave the hospital. It's widely seen as a "little snip"[2] that doesn't result in any major harm—a simple, low-risk procedure that removes a bit of "excess skin," both to enhance the esthetic appearance of the penis and to make it easier to wash and avoid infection. It is often assumed that future sexual partners will prefer a circumcised penis, and that "uncircumcised" youths may face teasing in the locker room.[3] Besides, in most cases, at least one of the child's parents or guardians will also have been circumcised shortly after birth, in accordance with the dominant norm. Perhaps children will be less confused, as they learn about their bodies, if their genitals have been surgically shaped so that they "look like" one of their parents "down there."[4]

Since there are several potential benefits, the thinking goes, both physical and psychosocial, and not too many serious harms to worry about—unless the surgery is "botched"[5]—why would circumcision be ethically controversial? Plausibly, if the practice were unethical in some way, it would not be performed on a routine basis by licensed physicians in one of the most medically advanced countries in the world. Besides, it is (also) an important religious custom for practicing Jews and Muslims. It might, therefore, be seen as anti-Semitic or Islamophobic even to suggest it could be morally wrong.[6] Better to leave well enough alone, this perspective concludes, and turn our attention to more pressing issues.

Other perspectives

Meanwhile, readers from Europe, Central or South America, Australasia, and much of Asia may also be puzzled about the need for a chapter on circumcision, but for a different reason. In these parts of the world, the vast majority of babies categorized as male at birth, just like those categorized as female at birth, do not have their genitals surgically modified before being sent home from the hospital.[7] Instead, children grow up and spend the rest of their lives with their sexual organs "intact"—that is, surgically unmodified—and most would never think of having them cut or altered unless it were strictly necessary. In these contexts, it is, rather, the moral badness of excising any part of a child's healthy sexual anatomy that is the default ethical position.

In these cultures, the penile foreskin is treated similarly to the labia minora, the latter of which, where joined at the top of the vulva, form the anatomically homologous clitoral foreskin or hood (see Box 1). Each of these genital structures is considered to be a valuable part of a person's sexual embodiment: protective, erogenous tissue that usually should not be cut into, much less permanently removed, unless there is a medical emergency.

Granted, like other parts of the body, male or female genital tissues can sometimes become irritated, inflamed, or infected—even cancerous. But doctors from non-circumcising cultures understand that these problems are either rare, or rarely serious, if one practices basic health and hygiene. Therefore, the mere possibility of future health problems involving the foreskin or labia is not typically seen as a legitimate reason to remove these tissues from someone who cannot consent.[8]

The child may, after all, grow up to prefer modes of disease prevention that are more effective, or less risky, than genital surgery. And if not, they will still have the option of surgery available for them to pursue on a consensual basis. By contrast, the inverse—the option of

genital intactness for someone who was surgically altered without their consent—does not exist, and this might be regarded as an ethically significant asymmetry.[9] In any case, if a health problem does arise relating to the penis or vulva, it can usually be safely and effectively treated (for example, with a topical cream or antibiotics) with no loss of tissue nor risk of surgical complications.[10]

A small minority of individuals, for personal reasons, may end up pursuing a non-therapeutic genital procedure—be it penile circumcision, clitoral hood reduction,[11] or elective labiaplasty—in line with their own preferences and values. But it is seen as obvious, in these cultures, that this should be an elective decision made by a fully informed, sufficiently autonomous person about their own body, rather than something imposed on a non-consenting child. The fact that genital cutting of children is practiced within certain religious communities only adds to the complexity of the issue.

Box 1. *The foreskin*.[12]

The genital foreskin or prepuce, its formal name, is a protective skin covering or sheath-like tissue that exists in all primates, including humans of both sexes. Thus, with rare exceptions, not only males, but also females are born with a foreskin (the female foreskin is also known as the clitoral prepuce or hood). In both cases, the foreskin shields the head of the phallus—the clitoral or penile glans—from abrasion, while also housing many specialized nerve endings. When the individual becomes sexually active, these nerve endings can, when appropriately stimulated, contribute to feelings of sexual pleasure.

The foreskin is not a discrete, separate structure, but is either continuous with the labia minora (in females) or the penile shaft skin (in males). It evolved to function with the rest of genital organ of which it is an integrated part. For example, the foreskin covers and protects the head of the penis or clitoris, except when it is retracted—for example, during a state of arousal—much as the eyelid covers and protects the eye unless it needs to be exposed. In each case, the covering structure preserves the softness and sensitivity of the part that is kept mostly internal, while forming a barrier against potential contaminants.

The penile foreskin is relatively large: it has a mean reported surface area of about 30 to 50 square centimeters in adults, which is roughly the size of a credit card. It is a highly elastic, lubrication-preserving tissue that typically glides back and forth during sex or masturbation—reciprocally to the movement of the shaft—and it is the most sensitive part of the penis to warmth and light touch. The clitoral foreskin, while smaller in absolute terms, is a significant sensory platform in its own right. As an extension of the similarly touch-sensitive, stretchable, self-lubricating labia minora, it plays an important role in female sexual enjoyment. For example, it allows the head of the clitoris to be stimulated through its own skin covering, rather than by direct contact (which can be unpleasant and even painful).

Regardless of sex, the foreskin exhibits considerable anatomical variation between individuals: some have a larger foreskin, while others have a smaller one; some have an acutely sensitive foreskin, while the foreskin of others may be less sensitive, and so forth. In the case of an infant, it is not possible to tell precisely how much their foreskin will contribute to their future sexual enjoyment—and thus what they stand to lose, experientially, if it is removed. Nevertheless, most sexually active persons with genital foreskins, male or female, appear to place a positive value on the tissue.

Finally, in some parts of Africa, South and Southeast Asia, and the Middle East, both males *and* females have their genitals cut for cultural reasons, and this is seen as only right and proper.[13] Often, the cutting occurs in the context of a rite of passage into adulthood, where girls, as much as boys, are expected to show courage and stoicism in the face of pain, thereby demonstrating their maturity.[14] However, there is much variation: depending on the specific culture and which forms of cutting it has adopted, the ritual for either sex can happen anywhere from infancy to late adolescence; it may or may not be associated with a religion, such as Islam or Christianity; it can happen in a big-city hospital or a rural village, by a trained medic or traditional cutter; and either the male or female procedure may be more risky or severe (see Box 2 for more information).

Parents in these cultures understand that their children will experience pain from the genital cutting and throughout the process of recovery. The parents themselves will have gone through a similar ordeal when they were younger. But they commonly believe that this will prepare the child for the harsh reality of adult life: the pain of childbirth, for example, or fighting in war.[15] They also know that their child, whether male or female, will not be considered eligible for marriage and other adult privileges until they are symbolically "purified," or prove their ability to withstand hardship as a costly signal of their commitment to the group.[16]

Finally, they know that "uncircumcised" children, both girls and boys, risk bullying or other mistreatment from their circumcised peers unless they succumb to the practice.[17] In such a context, these parents think, to *fail* to have their sons or daughters cut in the socially prescribed manner would put them at risk of serious social harm. For example, they may face rejection from potential sexual partners or be ostracized from the wider community.

In many of these contexts, such fears are legitimate for children of either sex. This can create a collective action problem whereby it is difficult for any single family to stop the practice of child genital cutting—even if they wanted to—without putting their child at a significant disadvantage. This is structurally similar to the problem of Chinese foot binding that once persisted for many centuries.[18] However, there is at least one striking difference between Chinese foot binding and genital cutting, in that foot binding was performed in a sex-discriminatory manner, only on girls to make them eligible for marriage. By contrast, depending on the culture, the practice of child genital cutting as a prerequisite for marriage or other social goods is almost always imposed either on girls and boys alike, or only on boys. There are virtually no cultures in which genital cutting is a socially enforced custom only for girls.[19]

Box 2. *Varieties of genital cutting: a cross-cultural perspective*.[20]

Both male and female genital cutting fall on a wide spectrum across cultures, with variable implications for health and sexuality. In parts of South and Southeast Asia, for example, primarily Muslim doctors or nurses perform circumcision in a gender-inclusive manner, seeing the procedure as both hygienic and religiously beneficial for children of either sex. In these contexts, the cutting is often done in a modern clinical environment. The typical practice for girls involves a "nick, abrasion, piercing or small cut restricted to the female foreskin" (see Box 1), often without removal of tissue but aiming to draw a ceremonial drop of blood. In many cases, the cut heals quickly, resulting in no visible change to the vulva. The practice for boys is more substantial: assuming a typical male circumcision, it involves the removal of most or all of the penile foreskin—about a third of the skin system of the penis—resulting in a larger wound that takes longer to heal and often leaving a permanent scar. In either case, the child may experience psychological distress from having their genitals exposed and pierced with a sharp object, typically while being held down by one or more adults, irrespective of how much tissue is removed.

Different forms of genital cutting are practiced in other cultures. In some parts of northeast Africa, for instance, the boys may be traditionally circumcised while the girls are subjected to infibulation: a much more severe procedure that involves cutting the labia minora and suturing the vulva, with or without also cutting the clitoral glans, leaving only a small outlet for urine and menstrual blood. This is typically excruciating and can be highly traumatic; and it carries a well-document risk of multiple complications, many of which can be serious or lifelong. Meanwhile, in parts of southern Africa, the girls do not undergo genital cutting, while the ritual for boys is characteristically harsh: in some groups, there is an extremely high rate of infection, accidental penile amputation, and even death—sometimes by suicide—following the male circumcision ritual, in which multiple boys may be cut with the same septic knife.

In both sexes, the physical risks of genital cutting vary in both likelihood and magnitude depending on multiple factors: the individual anatomy of the child, which specific tissues are damaged or removed, the severity of the cutting, the surface area of the resulting wound, and—most significantly—the extent to which the practice has been medicalized. This refers to the process by which a non-medical practice, like genital cutting, gets gradually taken over by healthcare professionals who may use, for example, sterilized instruments and pain control. Often this is done in the name of harm reduction, although it can also cause the practice to be seen as safer and more "modern," as a result of which it becomes further entrenched. For that reason, among those who believe that medically unnecessary genital cutting of a child violates the child's human rights, medicalization is sometimes opposed. Instead, it is argued that doctors and others should seek to stop the cutting from happening altogether.

Three patterns

To summarize, there are three main patterns of non-therapeutic (that is, medically unnecessary) genital cutting of children carried out across the globe. In the first pattern, the dominant culture endorses routine or religious genital cutting only of children categorized as male at birth and subsequently raised as boys ("boys"), as in the United States, Israel, Afghanistan, or Saudi Arabia, for instance. In these cultures, children categorized as female at birth and raised as girls ("girls") are ineligible for genital cutting, or may be seen as deserving protection from it.[21]

In the second pattern, neither boys nor girls of the dominant culture have their genitals cut, unless there is a strict medical indication—which in these contexts is rare. This is the prevailing pattern throughout Europe and indeed most other parts of the world. In the third pattern, *both* boys and girls of the dominant culture have their genitals cut for non-medical reasons, for example, in the context of a rite of passage or religious initiation. Typically, this occurs around puberty, but the timing ranges from right after birth to before marriage.

In this chapter, I present a moral argument in favor of the second, "European" pattern, both as a matter of ethics and gender equality. I do not discuss legal implications. In other words, I argue that, morally speaking, *neither* boys nor girls who are too young to consent should have their genitals cut—to any extent—unless it is medically necessary to do so. The alternative, from a gender equality perspective, would be to argue that *both* boys and girls should have their genitals cut on an equal basis, for example, by a doctor using sterile instruments. But from an ethical perspective, I argue, this would not be acceptable.

Nevertheless, it is a real concern. In fact, a growing number of bioethicists and legal scholars have argued in recent years that since medically unnecessary, non-consensual male genital cutting is currently permitted—even in so-called "Western" countries that purport to

uphold children's rights—it is discriminatory not to allow medically unnecessary, non-consensual female genital cutting in the same countries, as long as it is no more harmful than the male forms of cutting that are currently allowed.[22]

In practical terms, these scholars argue that it should be legal to cut off parts of a girl's healthy labia or clitoral hood without her consent, and that it should be considered ethical for doctors to do this—even without a valid medical diagnosis—if the parents request it for cultural or religious reasons. As one prominent scholar has put it, "If the practice is legal for the gander, why should it be banned for the goose?"[23]

I argue for the opposite moral position. I claim that *all* children who are insufficiently mature to give their own morally valid consent should be protected from genital cutting that is not medically necessary—no matter how harmful or harmless someone else judges the cutting to be.[24] In other words, as I have stated elsewhere, "ganders and geese may do as they please when it comes to interventions into their intimate anatomy. Goslings, by contrast, should be protected from medically unnecessary genital cutting until they are capable of making their own decision."[25]

Importantly, my argument does not only apply to children ("goslings" in this analogy) who are what is known as *endosex*—that is, children whose sexual anatomy is judged to fall within normative standards for binary male or female classification according to the criteria used by biologists.[26] Rather, it applies just as much to children who are *intersex*—that is, children whose sexual anatomy is judged to depart from those normative standards (for example, by having a mix of features that are typical for males and females due to different genetics or processes of growth and development in the womb).[27]

In most settings, children with intersex traits will be categorized as either female or male, even in cases of uncertainty. Often, this decision will be reinforced by risky or repeated surgical

interventions into their genitalia to try to approximate a binary endosex appearance. Although most of these children grow up to identify as members of the sex category to which they were assigned—and with their associated gender of rearing—some of them come to feel that their sex assignment was mistaken or incompatible with their gendered self-conception.[28] However, regardless of a person's sex characteristics or gender identity, I argue that the decision about whether a sharp instrument should be brought into contact with one's sexual anatomy, much less used to excise healthy tissue from one's genitalia, should be up to affected person when they can make their own informed decision—unless a medical emergency requires immediate intervention. In the course of making this argument, I hope to guide the reader through some of the main points of scholarly disagreement between defenders and critics of medically unnecessary child genital cutting.

A point on language and defining terms

Before getting into these arguments, I would like to raise a point about language. I was originally invited to write a chapter on "male circumcision," as the practice is commonly referred to, but I have not used that phrase in the chapter so far. I do employ the terms "male" and "female" to refer to culturally sexed body parts,[29] and "boy/man" or "girl/woman" to refer to presumed or actual occupants of socially recognized gender roles.[30] Indeed, genital cutting practices are often key cultural mechanisms by which children become dichotomously "gendered" in this social sense.[31] But I have tried to avoid using gender/sex language when making certain anatomical distinctions. There are several reasons for this.

First, it reinforces the male/female binary for body classification, which is exclusionary toward people with intersex traits (many of whom also have foreskins). Indeed, as mentioned previously, intersex children are regularly subjected to medically unnecessary genital surgeries—

some of them far more invasive than circumcision—in an attempt to force their otherwise healthy, functional anatomy to conform to this cultural binary.[32] Notably, these surgeries take place even in European and other cultures where neither boys nor girls (who are not intersex) undergo genital cutting. And yet, the consequences of intersex genital surgeries, including nerve damage, scarring, numbness, recurring infections, and other common complications, have been devastating for many intersex people.[33]

Second, the ethical arguments I make in this chapter rely on principles, such as informed consent, bodily integrity, human rights, and sexual autonomy, that apply to all humans regardless of their particular sex traits or gender identification. Indeed, to apply these principles selectively based solely on a person's sex or gender would be an unethical—and if put into practice, potentially illegal—form of discrimination.

Third, some people who undergo "male" circumcision—that is, removal of the penile foreskin—do not identify as boys or men in terms of their gendered self-conception.[34] Indeed, in a country such as Israel or the United States, the vast majority of transgender women (and a large proportion of non-binary individuals) will have had a significant part of their genitals removed soon after birth, through routine or religious penile circumcision. Not only may this affect these women's sense of bodily or sexual integrity, upon discovering what happened to them when they were infants (recall that the penile foreskin is anatomically analogous to, and shares many properties with, the labia and clitoral hood); but it may also pose a practical problem for the subset of these women who pursue certain gender-affirming surgeries.[35]

This point is worth dwelling on for a moment. Although many trans women neither need nor desire surgery to create a neovagina in order to feel fully themselves and/or comfortable in their own bodies, some do, and for them, the surgery may be necessary for their mental health or well-being.[36] The most common surgery for this purpose is a genital inversion technique that

makes ample use of the penile foreskin to create a vaginal lining and/or labia minora and clitoral hood.[37] The foreskin is ideal for this purpose because it is a highly elastic, hairless, richly innervated genital tissue (see Box 1) that comes equipped with its own blood supply.[38] If the foreskin has been removed through circumcision, however, the surgeon must resort to less-accessible shaft skin from further down the penis or even skin grafts from other parts of the body. (The same problem, incidentally, applies to burn victims who have been previously circumcised, who would otherwise have had their own foreskins available as donor tissue.)[39]

In short, it is not only cisgender (that is, non-transgender) boys and men, but also transgender girls and women, along with some non-binary and genderqueer individuals, who are affected by penile circumcision. I therefore aim to use anatomically accurate language (that is, "penile circumcision") rather than sex/gender-coded language (that is, "male circumcision").

Finally, because my argument will rest on the notion of medical necessity, I need to define the term. I consider an intervention into a bodily state to be medically necessary when (1) the bodily state, if left unmodified, poses a serious, time-sensitive threat to the person's well-being (typically due to a functional impairment in an associated somatic process), and (2) the intervention, as performed without delay, is the least harmful feasible means of changing the bodily state to one that alleviates the threat.[40] All other interventions are medically unnecessary according to this conception.

It is important to stress that the concept of medical necessity invoked here is meaningfully different from the concept of being (merely) "medically beneficial." The latter concept is often used to describe penile circumcision in the United States[41] (although not in Europe, Australasia, and so on).[42] It picks out a weaker standard for authorizing a proposed intervention, according to which it is morally permissible to infringe upon the bodily integrity of a non-consenting individual, just so long as the expected health-related benefits of the

infringement outweigh the expected health-related harms. In the following sections, I am going to explain why such "weighing" of expected health-related benefits and harms is not an appropriate mechanism for evaluating the permissibility of non-consensual genital cutting.

Benefits and harms

The usual way to argue against medically unnecessary genital cutting—especially if it is non-consensual—is to argue that it crosses a threshold of net physical harmfulness that makes it morally impermissible.[43] Otherwise, if a practice is deemed to fall within what is sometimes called the "zone of parental discretion,"[44] it is assumed that parents should be free to make decisions about what happens to their children's bodies.

By contrast, I am going to argue that it is always morally impermissible for an adult to touch, cut, or otherwise interfere with a child's so-called "private" anatomy, regardless of how physically harmful, harmless, or even beneficial the adult judges the interference to be. The only exception for interfering with a non-consenting person's genitalia, I argue, is for interventions that are medically necessary and so cannot, by any reasonable standard, ethically be delayed until the person is in a position to consent.

In other words, I suggest, we should not look at a non-consenting person's (healthy) sexual anatomy, even if this person is our child, and ask, "What level of net harm or benefit do I expect will result from surgically removing part of this person's genitalia?"

Rather, we should think: "Given that this person does not have any medical problems affecting their sexual anatomy, much less any that would justify exposing that part of their body to surgical risk, I am obligated to protect them from any such unnecessary genital operations until they can make their own decision."

One significant problem with the "weighing" approach is that, in the case of non-consensual genital cutting, the person(s) doing the weighing are not the same person as the one who will be permanently affected by the procedure if it occurs. The fact that someone is not currently able to make an informed decision about their own sexual anatomy does not, in general, license others—even one's parents—to make such decisions for them. Rather, as I will argue, the default position when it comes to other people's sexual anatomy is not to interfere.[45]

It is also hard to know what *kinds* of benefits and harms we should factor into our analysis. For example, consider a child who has had part of their genitals removed based on their parents' religious or cultural beliefs. This child may grow up to feel that the genital modification has brought them closer to God or their community. This would be an incalculable benefit to the child and might seem to justify modifying their genitals without first obtaining their consent. That way, there might be less risk of the child feeling alienated from God, or their community, during the period between birth and becoming sufficiently mature to opt for a consensual genital modification themselves.[46]

However, the same child might grow up to become an atheist or otherwise dissociate from the religious or cultural community into which they were born. Indeed, this is increasingly common in contemporary, multicultural societies, in which people move from place to place, have access to the internet, and in general are exposed to different points of view.[47] This child will, by contrast, have had one of their most intimate body parts irreversibly altered, on account of a controversial set of beliefs they do not share. Perhaps they have a scar on the organ that daily reminds them of this fact, which they find to be disturbing or distressing. This could be an incalculable harm to the child, who may experience what happened to their penis or vulva—when they were in a highly vulnerable condition, no less—as a violation of their bodily or sexual integrity.[48]

That these contrasting potential outcomes are not easily measurable scientifically does not, of course, mean that may not be of the utmost importance to a person's sense of dignity or overall well-being. Nevertheless, what these examples show is that whether a given genital modification *is* ultimately harmful or beneficial, on balance, will depend on the attitudes, beliefs, or preferences of the genitally modified person in the context of their social environment.[49] It cannot be assumed that they will adopt the beliefs or values of the majority within that environment; and nor can the concepts of harm or benefit be reduced to expected "physical" outcomes.

This leads to another problem with "weighing" harms and benefits. If we look at the entire circumstances of an individual's life, we may find that even a highly invasive, risky, and physically damaging genital operation, such as infibulation (see Box 2) could, given certain background social conditions, end up being net beneficial to the child (or as it might be said, "in her best interests"). For example, infibulation could be a net benefit to someone who believes, as is common in practicing cultures, that infibulation is necessary to become a proper woman; or if it makes her feel connected to her ancestors; if it allows her access to marriage and other valued cultural institutions from which she would otherwise be excluded; if it helps her avoid teasing and bullying from her infibulated peers, and so forth.[50]

Indeed, *any* bodily harm inflicted on a child, no matter how physically injurious, can in principle be justified by a "best interests" analysis if the social benefits of the injury—or social costs of keeping one's body intact—are sufficiently great. If you add potential *metaphysical* benefits or costs, such as might be associated with being perceived to follow or violate the will of a divine entity, the possibility that even a severe bodily injury could be in the overall "best interests" of a child (including their spiritual interests, to put that way) is increased.

What this shows is that parental judgments of the child's best interests, based invariably on the parents' own metaphysical beliefs and/or the social norms of their chosen or inherited community, cannot serve as a moral basis for determining the permissibility of subjecting their child to a medically unnecessary genital surgery. Rather, such a "best interests" approach *assumes* the permissibility of the genital surgery, otherwise the parents would not request it in the first place (unless they were, for example, sadists or psychopaths; but we can assume such a situation is rare).

Indeed, even setting aside purported spiritual or religious benefits, insofar as certain very real, measurable social benefits and harms are included in such a calculus, the non-consensual removal of healthy tissue from a child's genitals may in fact be justified on this basis. The reason for this is simple. Any society that endorses or perpetuates a set of cultural norms or religious precepts according to which a child's overall well-being is, functionally speaking, held hostage to a non-consensual genital surgery can, by that very fact, *make* the surgery "in their best interests." For example, if community members or religious leaders choose to exclude, punish, or ostracize a child who has not had the requisite tissue excised, non-consensually, from their genitalia; or if they choose to tolerate the exclusion, punishment, or ostracization of this child, it may well be in the best interests of the child to have that part of their sexual anatomy removed.

Moreover, it could even be negligent of the child's parents not to authorize the medically unnecessary genital surgery for their child at the first relevant opportunity. But this does not mean that such a genital surgery does not seriously wrong the child or violate their rights, for reasons I will outline below. Rather, it would imply that the society has a strong moral reason to change some of its discriminatory norms and practices. Rather than imposing unbearable social harms or costs on children whose parents do not want to participate in an institution that requires vulnerable children be genitally injured, societies should promote the unconditional love and

acceptance of all children by community members—whether or not those children's genitalia have been altered.

Adopting a medical focus

One potential solution to the problems highlighted in the previous section might be to consider the moral permissibility of genital modification from the perspective of medical ethics. According to this approach, whatever we might think about the appropriateness of using a "best interests" standard from the standpoint of society at large, in a medical context, doctors are bound to follow a more demanding set of ethical principles, given their position of authority, the vulnerability of many patients within their care, and the special role they have been assigned by society to intervene in people's bodies, sometimes in highly invasive ways.

Obviously, a doctor cannot evaluate the metaphysical beliefs or cultural commitments of the parents of a child within their care, nor even make a particularly sophisticated judgment about the social benefits or harms to which a child may be exposed as a result of getting—or not getting—a genital surgery. Those matters are beyond a doctor's expertise. What a doctor should be able to judge, however, are the *medical* implications of performing, or not performing, a surgery. And so, perhaps, we should consider the ethics of genital modification from the perspective of strictly health-related outcomes.

In other words, perhaps we can come to an agreement as to the *medical* benefits and harms of child genital surgery, and decide its permissibility on that basis. Suppose that we weigh the potential health-related benefits against the potential health-related harms, and we conclude that the potential benefits outweigh the potential harms. Would this make it permissible for a doctor to perform a genital surgery that does not pass the "medically necessary" threshold—defined previously—on someone who cannot consent?

I maintain that the answer is no. As I will argue, it is always impermissible to interfere with, much less perform surgery on, the sexual anatomy of someone who cannot consent, unless it is medically necessary to do so. I will make that argument in a subsequent section. But suppose that for now, we pursue this idea that a (merely) "medically beneficial" standard might in fact justify such a surgery.

Immediately a problem arises: Who, again, should do weighing here, and what are the weights they should assign? As it turns out, coming up with a ratio of benefits to harms in this area—even if we restrict ourselves to considering only the physical health of the individual—depends on numerous contested factors. This makes universal agreement, even among similarly qualified medical experts, hard to come by.[51]

For example, the American Academy of Pediatrics asserted in 2012 that the health benefits of newborn penile circumcision outweigh the risks,[52] by which they seemed to mean the known or estimated risk, based on limited data, of primarily short-term surgical complications. Their policy statement was strongly criticized by international medical experts.[53] In 2015, the Canadian Pediatric Society, looking at the same medical literature, concluded that the health benefits and risks were "closely balanced."[54] By 2020, the Danish Medical Association concluded that there was no compelling evidence of a "clear health benefit" for circumcision prior to sexual debut.[55]

Part of the problem is the generally poor quality of the scientific evidence. Even the strongest available evidence is far from conclusive. Consider the claim, based on data from three randomized clinical trials, that adult, voluntary circumcision in some African countries can reduce the risk of female-to-male transmission of HIV.[56] Because the "treatment" condition of these trials involved undergoing a genital surgery and having a substantial portion of one's penile skin removed, it was not possible to conceal this from participants. Thus, the most basic

requirements of a high-quality randomized trial—namely, that it be "double blind" (that is, neither the participants nor the researchers know what condition the participant has been assigned to until after data collection) and placebo controlled (that is, everything except the proposed causal mechanism is held constant between the two conditions)—was not even possible.

Nevertheless, there is evidence from a fourth trial that circumcision can *increase* the risk of male-to-female transmission of HIV by roughly the same amount.[57] This would suggest that women—who are already more vulnerable to HIV and AIDS in many African countries—may be put at risk by the penile circumcision of their partners.[58] In any case, both of these findings stemmed from epidemiological environments with very high rates of heterosexually transmitted HIV and a low baseline prevalence of penile circumcision. Thus, neither finding, even if were compelling on its own terms, could be simply copied and pasted onto non-voluntary circumcision of children in a country such as the United States.[59]

And that is the best-case scenario for causal evidence. Much of the rest of the health claims that are commonly raised for circumcision are based on correlational studies with messy outcomes and inadequate control of confounders.[60] No wonder scientists from different countries disagree. But even so: let us suppose, for the sake of argument, that scientists from the U.S., Canada, Denmark, and elsewhere could at least all agree on the health-based implications of the data. Suppose they agreed on which studies were well-conducted, and what those studies basically showed. Still, they would not be able to reach an "objective" consensus about the weights to assign to benefits or risks.

Assigning weights

To see this, consider the loss of the foreskin itself, a "risk" of circumcision that has close to a 100% chance of occurring. (Here we are talking about the penile foreskin, but the same argument applies to the clitoral foreskin or labia.) How much weight should be given to this outcome, and in what direction? If you place a positive value on having a foreskin, as most people who have one seem to do, then its loss is a harm to you even if there are no (other) surgical complications.[61] The likelihood of this loss, as noted, is close to 100%, and the magnitude (degree of badness or disvalue) of the loss scales with the degree to which you value the tissue. If you value it a lot, you will be greatly harmed by circumcision; if you value it a little, you will be less harmed—again, setting aside the risk of other complications.

If, by contrast, you place a net negative value on having a foreskin, as is apparently uncommon among those who have one, then its removal will count as a benefit to you. Perhaps this is one reason why people who were circumcised in infancy come to believe the foreskin is "just a bit of worthless skin." It helps maintain the subjective disvalue they place on having a foreskin, which is a state they cannot physically be in.[62] By contrast, someone who was not circumcised in childhood always has, as mentioned, the option of surgery available to them, so there would be less of a need—psychologically speaking—to place a negative value on having a foreskin.[63]

As a more general matter, the overall ratio of benefits to harms for penile circumcision, or any other form of genital cutting—and this remains true even if we focus only on health-related benefits or harms—will depend on such factors as the following:

> the specific weights assigned to the potential outcomes of the intervention, given, among other things, (a) the subjective value to the individual of the body parts that may be affected, (b) the individual's tolerance for different kinds or degrees of risk to which

those body parts may be exposed, and (c) any preferences the individual may have for alternative (e.g., less invasive or risky) means of pursuing the intended health-related benefits.[64]

In other words, there *is* no objective, purely scientific, or other supposedly "value-free" way of deriving such a ratio. No committee or panel of medical experts can decide the question for everyone else. To see this point by way of another example, consider the claim that routine penile circumcision in infants reduces their risk of getting a urinary tract infection (UTI). These data are contested, because there is evidence that UTIs are misdiagnosed more frequently in children with intact penises than in children with circumcised penises;[65] but let us just assume the claim is true. According to the American Academy of Pediatrics, which has the most favorable stance toward penile circumcision of all comparable medical bodies, it would take more than 100 circumcisions to prevent a single UTI, given the low absolute risk of UTIs in children with penises regardless of circumcision status.[66]

That means that more than 100 children would have to be exposed to surgical risk on their genitals, with the intended outcome of removing healthy tissue, in order to prevent 1 (unknown, statistical) child from acquiring a UTI. But most UTIs are treatable non-surgically, with antibiotics, as they are in children with vulvas (who get them much more frequently).[67] So how much weight should one put on this alleged health benefit—a reduced risk of getting a UTI? Different people may have different answers, but at least one reasonable answer would be "close to zero."

By contrast, we might think of a potential harm of circumcision that presumably everyone would agree is an unambiguous physical or health-related harm. Take the risk of accidentally amputating part or all of the penile glans. Typically, this will be more likely to

happen in the case of a newborn circumcision than a later adult circumcision—when the organ has reached its full size—because the very small head of a baby's penis can more easily be pulled into a circumcision clamp, inadvertently and without the operator being able to see this.[68] The absolute risk of this happening is not known, but there are many case reports showing that it does sometimes happen even in "medicalized" settings.[69] Let us assume the absolute risk is low—far less than .01%. What is the weight we should assign to this possible outcome of circumcision, in evaluating potential benefits and harms?

Again, different people will give different answers. But the degree of badness associated with having part or all of one's penile head cut off—along with the risks associated with trying to address this problem, such as stopping the bleeding or undergoing corrective surgery—is presumably something that most people would regard as being extremely high. Importantly, the actual risk of a given outcome is calculated as the likelihood of its happening (in this case, stipulated to be low or rare) multiplied by the badness of the outcome, if it were to occur.[70] Thus, it may be reasonable to place a great deal of weight on this potential outcome of circumcision, even if it is unlikely to happen, due to the magnitude of badness that most people would associate with it if it occurred.

Also, given a sufficiently large number of circumcisions—over a million in the U.S. each year—even relatively unlikely surgical complications are bound to happen in some individuals, who may be affected for the rest of their lives. Although this may be an acceptable risk for medically necessary or consensual surgeries, it is harder to accept, from an ethical perspective, for surgeries that are neither medically necessary nor consensual.[71]

Health benefits versus risks

Now consider that virtually all of the statistical health benefits attributed to non-consensual penile circumcision (primarily, a reduced risk of various infections or diseases) can

be achieved by means that do not involve genital surgery, both in terms of prevention and treatment.[72] These less risky, non-surgical alternatives do not require the loss of any tissue that a person might reasonably wish to experience intact. For example: regularly washing the genitals with mild soap and water, practicing safer sex strategies when one becomes sexually active, using the appropriate medications for treatment if a health problem does occur, and so on.

Recall that we are assigning weights to future, potential health benefits that have been attributed to a surgical procedure that itself necessarily introduces risk: in this case, risk that is concentrated on a part of the body that most people try especially hard to shield from risk throughout their lives. This proposed surgery is to be performed on someone who does not currently face any health problem relating to the relevant body part (and who is at no more risk than average of ever developing such a potential problem, relative to other people with intact genitalia); and this surgery is designed to remove healthy, functional tissue from that body part (see Box 1). All of this is to happen without the informed consent of the person who will live with the consequences.

In such a situation, the health benefits in question are only probabilistic (they may not apply to any individual person; this depends mostly on their future behavior) and these benefits can, moreover, be achieved more safely by alternative means that do not involve non-consensual surgery. What should we conclude? One reasonable conclusion is that we should assign very little weight—perhaps zero—to any such health benefit (specifically, as attained through non-consensual surgery).

By contrast, many of the risks of penile circumcision—potential excision of too much tissue, pathological narrowing of the urethral opening (meatal stenosis), keloid scar formation, development of skin tags or bridges, uncomfortable erections due to tightness of skin, and so forth—are not primarily risks to which the individual would be exposed anyway, that is, without

genital surgery.[73] Rather, it is the surgery itself that exposes them to these risks.[74] Therefore, when attempting to balance the potential benefits and harms of circumcision, it may seem reasonable to assign considerable weight to any such risks, even if they are unlikely to happen in absolute terms (assuming a qualified operator). This is because the risks are surgery-specific and can have highly negative consequences for person's well-being, some of which may be lifelong.

In short, the potential health-related benefits of non-consensual penile circumcision can almost all be achieved without circumcision (or with consensual circumcision). Therefore, very little, or perhaps no, weight should be assigned to the prospect of attaining those benefits through non-consensual surgery. By contrast, the potential health-related harms of non-consensual penile circumcision are not typically harms to which the individual would otherwise be exposed. Therefore, plausibly, significant weight should be assigned to the risk of those harms, as they stem from non-consensual surgery.

Timing

As a final consideration, it is sometimes argued that it is less risky, medically speaking, to perform a penile circumcision in infancy compared to later in life, so that delaying the surgery until the person can make their own decision is effectively to raise the cost (in terms of potential physical harms) of deciding to be circumcised. Thus, it is not as simple as saying that the circumcision decision can be left to the affected individual.[75]

There are several problems with this argument.

First, it assumes that the circumcision will happen either way, whereas in reality, the vast majority of non-circumcised persons do not go on to choose circumcision, even in some cultures where circumcision is a dominant social norm.[76] And yet, clearly, non-circumcision is even less medically risky than infant circumcision.

Second, any number of surgeries—for example, earlobe removal—might be less medically risky in infancy compared to later in life, but it must first be established that it is ethical to perform the surgery without the affected person's own consent. As the example of earlobe removal suggests, however, it is not normally permissible to surgically remove normal, healthy body parts from someone who cannot consent, irrespective of the relative risk profile that may be associated with performing the surgery at various different times of life.

Third, even if one accepts the commonly asserted lower risk for infant circumcision (about a 1.5% risk of complications according to a systematic review),[77] this risk is not very much lower, in absolute terms, than the risk that has been estimated for adult, consensual circumcision: between 1.7-3.8% in the large, randomized trials mentioned earlier.[78] Even according to one of the most adamant proponents of infant circumcision, "virtually all" of the complications observed in the adult trials were "mild or moderate and were effectively treated."[79]

But if that is the case, the allegedly higher risk of adult complications should be assigned very little moral weight: (a) the absolute likelihood of such complications is not much higher, if higher at all, than the likelihood of complications that has been estimated for infant circumcision under comparable conditions; and (b) almost all of the adult complications that have been documented to occur are reportedly minor and treatable.

In either case, the risk of complications is asserted to be "low" by proponents of circumcision. A low risk divided by any amount is still a low risk. Thus, the relatively lower risk attributed to infant circumcision, even if one accepts this contestable empirical claim,[80] is not of any great ethical significance (given the low absolute risk either way), whereas the inability of the infant to consent to the permanent alteration of their sexual anatomy is, on any view, of considerable ethical significance.

Summing up

In the previous sections, I argued that assigning weights to purported benefits or risks of penile circumcision is a highly subjective process, even if we attempt to restrict ourselves to the "purely" medical domain. The ultimate value of any trade-offs will be primarily rooted in the beliefs and attitudes of the affected person.[81] Given the extraordinary unreliability of attempting to predict a person's future beliefs and attitudes many years ahead of time, especially with respect to something as potentially idiosyncratic as how they feel about their own sexual anatomy, this might suggest that the decision about whether to have their healthy genitalia altered should be left to them.

In any case, different evaluators will reach different conclusions about the precise nature of the various trade-offs between potential or actual benefits or harms. That is why evaluating the ethics of circumcision in terms of a utilitarian medical calculus—exemplified by the "medically beneficial" standard—is not going to be up to the job. This does not mean that the standard is never appropriate for "proxy" medical decision-making for children. Indeed, the standard may well be appropriate for certain interventions into the body, where the benefits and harms are much easier to define and measure, such that there is very little disagreement among qualified experts as to what those definitions and measurements imply; for example, certain childhood vaccinations.[82] But I suggest that "medically beneficial" is not an appropriate standard for evaluating the ethics of cutting or removing healthy genital tissue from a non-consenting person.

This suggestion is based, in part, on the highly personal nature of the genitals and their special significance to a person's sense of self, sexuality, sexual identity, and self-esteem.[83] Vaccinations do not raise similar issues. Indeed, if there were a vaccine, even a highly effective one, that could only be administered through a process that, for whatever reason, resulted in the permanent loss of one's labia or clitoral hood, say, it would *not* be approved for use in non-consenting persons. As my colleagues and I have argued:

> If someone is capable of consenting to genital cutting but declines to do so, no type or degree of expected benefit, health-related or otherwise, can ethically justify the imposition of such cutting. If, by contrast, a person is *not even capable of consentin*g due to a temporary lack of sufficient autonomy (e.g., an intoxicated adult or a young child), there are strong moral reasons in the absence of a relevant medical emergency to wait until the person acquires the capacity to make their own decision.[84]

Thus, I suggest, it is the informed consent of the affected person—not a subjective judgment of relative harmlessness made by a third party—that makes medically unnecessary genital cutting permissible as opposed to an impermissible bodily assault.[85]

Children and consent

Against the "consent" view I have just laid out, it might be said that very young children are not considered capable of autonomously consenting to any significant interference with their body; thus, their parent or guardian must provide "proxy" consent, not only to genital cutting, but to everything else. It is true that parents are rightly entrusted with making many decisions about their children's bodies. However, the inability of a child to provide morally valid consent to various actions taken toward them, does not entitle adults, even parents or doctors, to engage with the child's body in whatever way they please.

Consider the case of a doctor who performs a medical exam on a child's healthy sexual anatomy, knowing that the exam is not medically necessary for diagnosing or treating any problem. Or consider a parent who continues washing their child's genitalia long after the child becomes capable of doing this on their own. Even if it is not (intended to be) explicitly sexual,

something about an older person in a position of power touching the child's genitals without needing to do so strikes many people as morally troubling.

Indeed, it is the child's heightened vulnerability—their *inability* to use consent as a shield against unwanted intrusions into their body, and in particular into their sexual anatomy—that makes it so important to set a baseline moral expectation: namely, that *any* adult contact with a child's sexual anatomy is presumptively inappropriate. It only becomes appropriate under well-defined, limited conditions under which the avoidance of such contact (for example, failing to change a child's dirty diapers) would clearly be the greater wrong. Thus, I conclude, the *only* time it is permissible for an adult to touch, much less cut or remove tissue from, a child's sexual anatomy, is when it is medically necessary to do so.

Look at the matter this way. When it comes to engaging with the sexual anatomy of someone who is temporarily non-autonomous—because they are intoxicated, asleep, or a child—there are two types of error one can make. In the first type of error, one *fails* to engage with the person's sexual anatomy when, in fact, the person would have consented to, and even benefitted from, the engagement with their genitalia if they had been able to consent at the time. There is some loss here—a "missed opportunity" to benefit the person—but in most situations, the harm done, if any, is relatively small.

In the second type of error, one *engages* with a non-autonomous person's sexual anatomy, perhaps believing that this is what the person would consent to (or benefit the most from), when in fact the person *would not* have consented to the engagement had they been able to do so. In contrast to the first type of error, the potential harms to the individual associated with the second type of error—for example, a feeling of having been sexually violated, or of having had one's most important boundaries not respected—are enormous.[86]

Thus, it is much worse, from a moral perspective, to commit the second type of error compared to the first. Given any uncertainty about what the person would consent to in such a situation, therefore, it is morally imperative to refrain from engaging with their sexual anatomy. It is better to make the first type of error rather than the second.

The specialness of medical necessity

Against the view I have just laid out, someone might argue that too much weight is being placed on the notion of "medical necessity" as a threshold criterion for ethically touching or cutting the genitals of a non-consenting person such as a child. Why not also make an exception for touching or cutting that is perceived (by some) to be "culturally necessary" or perhaps "religiously necessary," for instance? In other words, what is ethically significant about *medical* necessity, in particular, such that it should serve as the only valid exception to an otherwise generally stated moral rule?[87]

This is my reply. It is widely appreciated that touching, cutting into, or removing healthy tissue from, another person's sexual anatomy is one of the most profound possible ways of engaging with their embodied personhood.[88] After all, in many cultures, the genitalia in particular are constructed as being almost uniquely private or personal, tightly linked to one's sense of self and self-esteem. Under ordinary conditions, engaging with someone's sexual anatomy should only be done if the person has (1) given their consent, or (2) there is no other reasonable option from the perspective of respecting their rights and (future) capacity to set and maintain important personal boundaries.

In the case of medically necessary surgeries, there is no other reasonable option. Here is an explanation of why. In the context of proxy decision-making, especially on behalf of highly vulnerable others, we must recognize that the values and motivations that underpin medically necessary surgeries—for example, not dying prematurely or becoming permanently disabled—

are of a very special kind. They are: (1) effectively universal, regardless of one's cultural background or metaphysical beliefs,[89] (2) typically among the strongest values and motivations that people have, and (3) highly robust against possible changes in perspective upon exposure to other points of view.

Thus, we can be practically certain, short of a signed advance directive or other similar document instructing otherwise, that the person *would* consent to the surgery if they were in a position to do so. It is this exceptionally high, warranted degree of certainty that the person would consent to a medically necessary procedure involving their genitalia (specifically, at a time prior to their being able to make their own decision) that entitles a proxy decision-maker to authorize something that would otherwise constitute a gross violation of the vulnerable person's sexual embodiment.[90]

Just as a parent cannot validly authorize a sexual assault for their child—even if the assault is not likely to be physically harmful, even if the assailant is not personally motivated by sexual desire for the child, even if the child is unconscious or too young to remember, and even if it is sincerely believed that the child "wants it" or "will be grateful for it one day"—so too is it the case that a parent cannot validly authorize an even more extreme intrusion into their child's sexual anatomy, again, regardless of the aforementioned factors (*a fortiori*).

This is especially the case given that the intrusion in question—genital surgery—is in fact physically harmful (it is painful, involves the removal of healthy tissue, creates a wound that may become infected, and so on) and leaves a lasting mark on the body. Therefore, even if the child was unconscious during the procedure, or too young to remember it happening, they will still carry the evidence of what occurred on their personal anatomy for the rest of their lives.[91]

Now consider the values and motivations that underpin medically unnecessary surgeries: for example, subjective esthetic preferences, commitment to a strict gender binary, promotion of

group solidarity, devotion to a particular god or gods, or a belief that it is okay to use non-consensual surgery to promote someone's health and hygiene when there are less risky and/or consensual methods that could effectively be used instead. These values and motivations are: (1) far from universal, varying greatly across cultural groups and metaphysical belief systems, (2) not necessarily among the strongest values and motivations that people have, and (3) highly susceptible to possible changes in perspective upon exposure to other points of view.

What is significant about medical necessity, then—as the threshold condition for permissibly performing a non-consensual genital surgery—is that it represents a rare case in which we can be sufficiently (and justifiably) certain that a temporarily non-autonomous person *would* consent to the engagement with their sexual anatomy. And by the same token, we can be equally sure that to *fail* to engage with their sexual anatomy by means of surgery under these conditions of medical necessity—thereby exposing them to a significant risk of death or disability—would be the greater wrong.

By contrast, genital surgeries that are perceived by some to be religiously or culturally necessary cannot clear that same high bar. Nor can surgeries that are claimed to be (merely) "medically beneficial." Again, even if a person agreed that certain alleged religious, spiritual, or even health benefits were extremely impressive, they might still prefer to pursue them in a way that did not involve genital surgery. According to the analysis I have provided, that would be their right.

Conclusion

The upshot is that we should not evaluate the ethics of child genital cutting by attempting a utilitarian calculus of benefits and harms. Invariably, parents who choose genital cutting for their children judge—however contestably—that the benefits, whether physical, psychosocial, or spiritual, outweigh the potential harms. And given certain background social conditions, they

may in fact be correct, even in the case of genital procedures, such as infibulation in high-prevalence societies, that are widely held (outside of those societies) to violate human rights.

Of course, some parents might be persuaded to adjust their weights in light of new evidence—and some may even reverse the balance, coming to conclude that genital cutting is, after all, a net harm to their child, especially if social conditions change. But the whole approach is mistaken, I have argued. Instead, insofar as a society recognizes the concept of sexual assault, defined as the medically unnecessary, non-consensual touching or penetration of another's sexual anatomy;[92] and insofar as a society purports to have a special concern for protecting vulnerable persons, such as children, from such putative violations; and insofar as children within a society are socialized to believe that their genitalia should be "off limits" to non-consensual touching or penetration by more powerful others, apart from certain well-defined situations,[93] there should an overwhelming presumption that it is a serious wrong to interfere with a child's sexual anatomy in any way, unless it is medically necessary to do so.

A surprising implication of this view is that routine newborn penile circumcision, as practiced in the United States, for instance, constitutes an extraordinary moral wrong playing out over a massive scale: as mentioned, over a million infants in that country undergo a medically unnecessary, non-consensual genital surgery each year. Moreover, this happens on an explicitly—that is, legally enshrined—sex-discriminatory basis: whereas no medically unnecessary genital cutting, however superficial, is legally permitted for children born with vulvas (endosex females), such cutting is widely practiced on non-consenting children born with penises (endosex males), as well as many children born with intersex traits.[94]

That such a grave moral problem, at such a significant scale, could be playing out on a daily basis in "modern" U.S. hospitals might seem to be unlikely or absurd. And yet, I have suggested that removing part of someone's sexual anatomy without their consent—in the

absence of relevant medical emergency—is an especially serious violation of their embodied personhood. If so, why do we not see thousands of supposed victims taking to the streets, protesting for their rights?

I have two responses here. First, there has, in fact, been a substantial global protest movement, led primarily by self-described victims or survivors, against medically unnecessary genital cutting of endosex males, endosex females, and intersex children for several decades.[95] The protests against female genital cutting have, fortunately, been highly visible for quite some time now, although efforts to stop the practices in high-prevalence countries have not yet been sufficiently successful. Protests against intersex genital cutting, long sidelined or ignored, have recently become more visible.[96]

The protests against male genital cutting have been no less fervent, but there has not been as receptive an audience. According to patriarchal gender norms, male bodies are expected to be tough and strong; they are not regarded as being as vulnerable as female bodies, or as susceptible to harm or violation, even in infancy.[97] And males who, once grown, discover what happened to them as infants, and who object to this and attempt to speak out, also find themselves in conflict with their prescribed gender role: expressing emotions such as sadness or grief, or sharing that one feels harmed or diminished, is prohibited by dominant norms of masculinity.[98]

Typically, in patriarchal cultures with a majority custom of penile circumcision, like the United States, males who speak out and assert that they feel harmed are often ridiculed, derided, and dismissed.[99] The same, of course, is true of women who speak out in cultures with a majority custom of female genital cutting. However, in the latter case, the cultures in question do not have as much power to shape the global discourse, which allows the voices of the protesting minority to be amplified by sympathetic outsiders.[100] Meanwhile, intersex people who have spoken out for decades have, until recently, received very little acknowledgement, much less adequate response.

In the context of significant power asymmetries in which systemic violations of bodily or sexual integrity occur, these violations are almost always treated as normal by the more powerful group who enacts them, so that their status as violations can be hard to discern. An obvious example would be the legal invisibility of marital rape until recently.[101] Children are the most vulnerable members of society, and it is therefore not surprising that widespread violations of their bodily or sexual integrity might be carried out by more powerful actors—in this case, adults, including doctors—without these violations being widely recognized.

That, of course, is the point of consciousness-raising. However, children are especially poorly positioned to engage in such activities, as they are taught to trust that whatever adults say, do, or authorize is presumptively appropriate or correct. And by the time most children become adults, if they have ever had an inkling that something wrong might have been done to them when they were little, it will typically be in their psychological best interests to "get over"—or form an adaptive preference for—whatever has already happened, given that it cannot be reversed.[102] And so the violations continue.

I am not arguing that there should, at this point in the cultural conversation, be criminal penalties applied to parents, doctors, or religious circumcisers who cannot be expected to know that parental "proxy" consent to medically unnecessary penile circumcision, just like parental "proxy" consent to medically unnecessary "female circumcision" (cutting of the clitoral foreskin), might not be legitimate.[103] I am only here concerned with making moral arguments, not legal ones, and my intention is to persuade, not to coerce.

However, as I have just alluded to, it is already widely understood that parents in the United States and similar countries cannot validly provide "proxy" consent to medically unnecessary cutting of the labia minora or clitoral foreskin of a non-consenting child, even if the parents believe this is a religious obligation. Given this, it will not be long before the

inconsistency of allowing non-consensual cutting of the penile foreskin (for any reason or no reason; currently none is required) will have to be dealt with more directly.[104]

[1] CDC, "Trends in Circumcision among Male Newborns Born in U.S. Hospitals: 1979–2010" (U.S. Centers for Disease Control and Prevention, 2015), https://www.cdc.gov/nchs/data/hestat/circumcision_2013/circumcision_2013.htm; WPR, "Circumcision by Country 2021" (World Population Review, 2021), https://worldpopulationreview.com/country-rankings/circumcision-by-country.

[2] Peter Aggleton, "'Just a Snip'? A Social History of Male Circumcision," *Reproductive Health Matters* 15, no. 29 (2007): 15–21.

[3] In reality, individual preferences for surgically altered versus unaltered genitals in a potential mate vary widely, both within and between cultures that have different rates of genital modification. As for locker room teasing, a study on this question conducted in the American Midwest suggests that it is penile size, not circumcision status that drives most penis-related locker-room teasing, and that teasing about non-circumcised penises, when it does occur, does not typically cause significant distress. Alexander E. Siobhan, Douglas W. Storm, and Christopher S. Cooper, "Teasing in School Locker Rooms Regarding Penile Appearance," *Journal of Urology* 193, no. 3 (2015): 983–88, https://doi.org/10.1016/j.juro.2014.09.105.

[4] For example, in having a similar scar line or exposed penile head (the size, hairiness, and other aspects of the organ will typically differ either way). It is unknown how often children see and compare their penis to that of a parent, or whether non-circumcised children of circumcised parents do indeed notice the difference in foreskin status—or if they do, whether they find it confusing or detrimental to well-being. Nevertheless, this common concern about a same-sex parent-child "mismatch" in North America is one of the main reasons parents give, when surveyed, for why they authorize newborn penile circumcision: they think it should be "like father, like son." The practice thus has a certain self-sustaining nature over the generations. See Sarah E. Waldeck, "Social Norm Theory and Male Circumcision: Why Parents Circumcise," *The American Journal of Bioethics* 3, no. 2 (2003): 56–57, https://doi.org/10.1162/152651603766436261; Chris Rediger and Andries J. Muller, "Parents' Rationale for Male Circumcision," *Canadian Family Physician* 59, no. 2 (2013): e110–15, https://www.cfp.ca/content/59/2/e110.

[5] Annette Schröder et al., "Serious and Fatal Complications after Neonatal Circumcision," *European Urology Focus*, 2021, online ahead of print, https://doi.org/10.1016/j.euf.2021.12.005.

[6] For important context on this worry, see Jenny Goodman, "Jewish Circumcision: An Alternative Perspective," *BJU International* 83, no. S1 (1999): 22–27.

[7] WPR, "Circumcision by Country 2021" (World Population Review, 2021), https://worldpopulationreview.com/country-rankings/circumcision-by-country.

[8] Morten Frisch et al., "Cultural Bias in the AAP's 2012 Technical Report and Policy Statement on Male Circumcision," *Pediatrics* 131, no. 4 (2013): 796–800, https://doi.org/10.1542/peds.2012-2896; see also, for example, RACP, "Circumcision of Infant Males" (Royal Australasian College of Physicians, 2010), https://www.racp.edu.au/docs/default-source/advocacy-library/circumcision-of-infant-males.pdf; BMA, "Nontherapeutic Male Circumcision (NTMC) of Children - Practical Guidance for Doctors" (British Medical Association, 2019), https://www.bma.org.uk/advice/employment/ethics/children-and-young-people/non-therapeutic-male-circumcision-of-children-ethics-toolkit/; DMA, "Lægeforeningens Politik Vedrørende Omskæring Af Drengebørn Uden Medicinsk Indikation" (Danish Medical Association, 2020), https://www.laeger.dk/omskaering-af-drenge-uden-medicinsk-indikation-er-etisk-uacceptabelt; FMA, "Poikien Ympärileikkaus | Lääkäriliitto - Lääkärin Etiikka -Kirja" (Finnish Medical Association, 2021), https://www.laakariliitto.fi/laakarin-etiikka/lapset-ja-nuoret/poikien-ymparileikkaus/; KNMG, "Nontherapeutic Circumcision of Male Minors" (Royal Dutch Medical Association KNMG, 2010), http://www.circumstitions.com/Docs/KNMG-policy.pdf, http://www.circumstitions.com/Docs/KNMG-policy.pdf. For a review of medical society attitudes toward labial modification in minors, see Andreas Kalampalikis and Lina Michala, "Cosmetic Labiaplasty on Minors: A Review of Current Trends and Evidence," *International Journal of Impotence Research*, 2021, online ahead of print, https://doi.org/10.1038/s41443-021-00480-1.

[9] For an in-depth discussion of the ethical relevance of this asymmetry, see Brian D. Earp and Robert Darby, "Circumcision, Sexual Experience, and Harm," *University of Pennsylvania Journal of International Law* 37, no. 2-online (2017): 1–57, https://www.researchgate.net/publication/315763686.

[10] For example, most cases of true phimosis (pathological inability to retract the penile foreskin) can be treated with a topical steroid, or in especially difficult cases, with a tissue-sparing preputioplasty; a full circumcision is rarely needed. See Anna Orsola, Jorge Caffaratti, and Jose Maria Garat, "Conservative Treatment of Phimosis in Children Using a Topical Steroid," *Urology* 56, no. 2 (2000): 307–10, https://doi.org/10.1016/S0090-4295(00)00576-8; Sesi Hotonu et al., "Save the Foreskin: Outcomes of Preputioplasty in the Treatment of Childhood Phimosis," *The Surgeon* 18, no. 3 (2020): 150–53, https://doi.org/10.1016/j.surge.2019.08.004.

[11] This procedure is also sometimes called "clitoral unhooding" and is often paired with labiaplasty and is intended as a cosmetic enhancement. See Steven P Davison, Kylie D Hayes, and Gabrielle LaBove, "The Extended Clitoral-U Suspension and Unhooding," *Plastic Surgery* 3, no. 1916 (2018): 1–2.

[12] This Box is loosely adapted from Brian D. Earp, "Against Legalizing Female 'Circumcision' of Minors," *Global Discourse*, 2022, online ahead of print, https://doi.org/10.1332/204378921X16347905414226. All source material for empirical claims is referenced within that paper. For more information, see Mohamed A. Baky Fahmy, *Normal and Abnormal Prepuce* (Cham: Springer International Publishing, 2020), https://doi.org/10.1007/978-3-030-37621-5; Christopher J. Cold and John R. Taylor, "The Prepuce," *BJU International* 83, no. S1 (1999): 34–44; and John R. Taylor, Anthony P. Lockwood, and A. J. Taylor, "The Prepuce: Specialized Mucosa of the Penis and Its Loss to Circumcision," *British Journal of Urology* 77, no. 2 (1996): 291–95, https://doi.org/10.1046/j.1464-410X.1996.85023.x. See also Vincenzo Puppo, "Anatomy and Physiology of the Clitoris, Vestibular Bulbs, and Labia Minora with a Review of the Female Orgasm and the Prevention of Female Sexual Dysfunction," *Clinical Anatomy* 26, no. 1 (2013): 134–52, https://doi.org/10.1002/ca.22177.

[13] Jasmine Abdulcadir et al., "Seven Things to Know about Female Genital Surgeries in Africa," *Hastings Center Report* 42, no. 6 (2012): 19–27.

[14] Harriet D. Lyons, "Genital Cutting: The Past and Present of a Polythetic Category," *Africa Today* 53, no. 4, (2007): 3–17. For a discussion of this, with further key references and ethical implications, see Brian D. Earp, "Between Moral Relativism and Moral Hypocrisy: Reframing the Debate on 'FGM,'" *Kennedy Institute of Ethics Journal* 26, no. 2 (2016): 105–44, https://doi.org/10.1353/ken.2016.0009.

[15] Alice Schlegel and Herbert Barry, "Pain, Fear, and Circumcision in Boys' Adolescent Initiation Ceremonies," *Cross-Cultural Research* 1, no. 1 (2017): 1–29, https://doi.org/10.1177/1069397116685780; Erika M. Baron and Florence L. Denmark, "An Exploration of Female Genital Mutilation," *Annals of the New York Academy of Sciences* 1087, no. 1 (2006): 339–55, https://doi.org/10.1196/annals.1385.018; Claire C. Robertson and Stanlie M. James, *Genital Cutting and Transnational Sisterhood: Disputing U.S. Polemics* (Chicago: University of Illinois Press, 2002).

[16] Thandisizwe R. Mavundla et al., "How Boys Become Dogs: Stigmatization and Marginalization of Uninitiated Xhosa Males in East London, South Africa," *Qualitative Health Research* 20, no. 7 (2010): 931–41, https://doi.org/10.1177/1049732310363804; Lindiswa Jan, "How Xhosa Women Play 'Hidden' Ulwaluko Role," September 11, 2017, https://www.iol.co.za/capeargus/opinion/how-xhosa-women-play-hidden-ulwaluko-role-11165772; Raven Rowanchilde, "Male Genital Modification: A Sexual Selection Interpretation," *Human Nature* 7, no. 2 (1996): 189–215, https://doi.org/10.1007/BF02692110; Rigmor C. Berg and Eva Denison, "A Tradition in Transition: Factors Perpetuating and Hindering the Continuance of Female Genital Mutilation/Cutting (FGM/C) Summarized in a Systematic Review," *Health Care for Women International* 34, no. 10 (2013): 837–59, https://doi.org/10.1080/07399332.2012.721417.

[17] Jo Boyden, Alula Pankhurst, and Yisak Tafere, "Child Protection and Harmful Traditional Practices: Female Early Marriage and Genital Modification in Ethiopia," *Development in Practice* 22, no. 4 (June 1, 2012): 510–22, https://doi.org/10.1080/09614524.2012.672957; WHO, "Traditional Male Circumcision among Young People" (Geneva, Switzerland: World Health Organization, 2009).

[18] Gerry Mackie, "Ending Footbinding and Infibulation: A Convention Account," *American Sociological Review* 61, no. 6 (1996): 999–1017, https://doi.org/10.2307/2096305.

[19] Abdulcadir et al., "Seven Things to Know about Female Genital Surgeries in Africa."

[20] The quoted description of female "circumcision" in this Box is from Richard A. Shweder, "The Prosecution of Dawoodi Bohra Women: Some Reasonable Doubts," *Global Discourse*, 2021, online ahead of print, https://doi.org/10.1332/204378921X16141809582432.The rest of the Box is loosely adapted from material in Arianne Shahvisi and Brian D. Earp, "The Law and Ethics of Female Genital Cutting," in *Female Genital Cosmetic Surgery: Solution to What Problem?*, ed. Sarah M. Creighton and Lih-Mei Liao (Cambridge: Cambridge University Press, 2019), 58–71. References for empirical claims are cited therein. See also Brian D. Earp and Rebecca Steinfeld, "Gender and Genital Cutting: A New Paradigm," in *Gifted Women, Fragile Men*, ed. T. G. Barbat,

Euromind Monographs 2 (Brussels: ALDE Group-EU Parliament, 2017). For further discussion, see Zachary T. Androus, "Critiquing Circumcision: In Search of a New Paradigm for Conceptualizing Genital Modification," *Global Discourse* 3, no. 2 (2013): 266–80, https://doi.org/10.1080/23269995.2013.813282. Regarding medicalization, see Bettina Shell-Duncan, "The Medicalization of Female 'Circumcision': Harm Reduction or Promotion of a Dangerous Practice?," *Social Science & Medicine* 52, no. 7 (2001): 1013–28, https://doi.org/10.1016/S0277-9536(00)00208-2; David L. Gollaher, "From Ritual to Science: The Medical Transformation of Circumcision in America," *Journal of Social History* 28, no. 1 (1994): 5–36; Frederick Hodges, "A Short History of the Institutionalization of Involuntary Sexual Mutilation in the United States," in *Sexual Mutilations*, ed. George C. Denniston and Marilyn Fayre Milos (New York: Springer US, 1997), 17–40, https://doi.org/10.1007/978-1-4757-2679-4_2.

[21] On the ineligibility of girls to undergo circumcision in certain cultures, where this is considered to be a valued social practice from which females are to be excluded based on their lower status under patriarchy, see, for example, the discussion in Shaye JD Cohen, "Why Aren't Jewish Women Circumcised?," *Gender & History* 9, no. 3 (1997): 560–78, https://doi.org/10.1111/1468-0424.00076.

[22] AAP, "Ritual Genital Cutting of Female Minors," *Pediatrics* 125, no. 5 (2010): 1088–93, https://doi.org/10.1542/peds.2010-0187; Richard A. Shweder, "The Goose and the Gander: The Genital Wars," *Global Discourse* 3, no. 2 (2013): 348–66, https://doi.org/10.1080/23269995.2013.811923; Kavita Shah Arora and Allan J. Jacobs, "Female Genital Alteration: A Compromise Solution," *Journal of Medical Ethics* 42, no. 3 (2016): 148–54, https://doi.org/10.1136/medethics-2014-102375; Allan J. Jacobs and Kavita Shah Arora, "Punishment of Minor Female Genital Ritual Procedures: Is the Perfect the Enemy of the Good?," *Developing World Bioethics* 17, no. 2 (2017): 134–40, https://doi.org/10.1111/dewb.12135; Raphael Cohen-Almagor, *Just, Reasonable Multiculturalism* (Cambridge: Cambridge University Press, 2021); Iddo Porat, "The Starting at Home Principle: On Ritual Animal Slaughter, Male Circumcision and Proportionality," *Oxford Journal of Legal Studies* 41, no. 1 (2021): 30–58, https://doi.org/10.1093/ojls/gqaa027.

[23] Shweder, "The Prosecution of Dawoodi Bohra Women."

[24] Some of the main articles in which my colleagues and I have advanced arguments for this view are: Brian D. Earp, "The Ethics of Infant Male Circumcision," *Journal of Medical Ethics* 39, no. 7 (2013): 418–20, https://doi.org/10.1136/medethics-2013-101517; Brian D. Earp, "Female Genital Mutilation and Male Circumcision: Toward an Autonomy-Based Ethical Framework," *Medicolegal and Bioethics* 5, no. 1 (2015): 89–104, https://doi.org/10.2147/MB.S63709; Brian D. Earp, "In Defence of Genital Autonomy for Children," *Journal of Medical Ethics* 42, no. 3 (2016): 158–63, https://doi.org/10.1136/medethics-2015-103030; Brian D. Earp and David M. Shaw, "Cultural Bias in American Medicine: The Case of Infant Male Circumcision," *Journal of Pediatric Ethics* 1, no. 1 (2017): 8–26, https://www.researchgate.net/publication/316527603; Brian D. Earp and Rebecca Steinfeld, "Genital Autonomy and Sexual Well-Being," *Current Sexual Health Reports* 10, no. 1 (2018): 7–17, https://doi.org/10.1007/s11930-018-0141-x; Alex Myers and Brian D. Earp, "What Is the Best Age to Circumcise? A Medical and Ethical Analysis," *Bioethics* 34, no. 7 (2020): 645–63, https://doi.org/10.1111/bioe.12714; Brian D. Earp, "Protecting Children from Medically Unnecessary Genital Cutting without Stigmatizing Women's Bodies: Implications for Sexual Pleasure and Pain," *Archives of Sexual Behavior* 50, no. 1 (2021): 1875–85, https://doi.org/10.1007/s10508-020-01633-x; Brian D Earp, "Male or Female Genital Cutting: Why 'Health Benefits' Are Morally Irrelevant," *Journal of Medical Ethics* 47, no. 12/e92 (2021): 1–10, https://doi.org/10.1136/medethics-2020-106782; Brian D Earp et al., "The Need for a Unified Ethical Stance on Child Genital Cutting," *Nursing Ethics* 28, no. 7–8 (2021): 1294–1305, https://doi.org/10.1177/0969733020983397. See also classic and recent work by: LeYoni Junos, "Bodily Integrity for Both: The Obligation of Amnesty International to Recognize All Forms of Genital Mutilation of Males as Human Rights Violations" (Hamilton, Bermuda: Amnesty International Bermuda, 1998), http://circumcisionharm.org/images-circharm.org/1998%20AI%20Report-Bodily%20Integrity.pdf; Godfrey B. Tangwa, "Circumcision: An African Point of View," in *Male and Female Circumcision*, ed. George C. Denniston, Frederick M. Hodges, and Marilyn F. Milos (Boston, MA: Springer, 1999), 183–93, https://doi.org/10.1007/978-0-585-39937-9_12; Nahid F. Toubia, "Evolutionary Cultural Ethics and the Circumcision of Children," in *Male and Female Circumcision*, ed. George C. Denniston, Frederick Mansfield Hodges, and Marilyn Fayre Milos (Boston, MA: Springer, 1999), 1–7, https://doi.org/10.1007/978-0-585-39937-9_1; Hanny Lightfoot-Klein et al., "Genital Surgeries on Children below an Age of Consent," in *Psychological Perspectives on Human Sexuality*, ed. Lenore T. Szuchman and Frank Muscarella (New York: John Wiley & Sons, 2000), 440–79; Christine Mason, "Exorcising Excision: Medico-Legal Issues Arising from Male and Female Genital Surgery in Australia.," *Journal of Law and Medicine* 9, no. 1 (2001): 58–67, http://europepmc.org/article/med/12116672; J. Steven Svoboda, "Promoting Genital Autonomy by Exploring Commonalities between Male, Female, Intersex, and Cosmetic Female Genital Cutting," *Global Discourse* 3, no. 2 (2013): 237–55, https://doi.org/10.1080/23269995.2013.804757; Arianne Shahvisi, "Cutting Slack and Cutting

Corners: An Ethical and Pragmatic Response to Arora and Jacobs' 'Female Genital Alteration: A Compromise Solution,'" *Journal of Medical Ethics* 42, no. 3 (2016): 156–57, https://doi.org/10.1136/medethics-2015-103206; Kai Möller, "Male and Female Genital Cutting: Between the Best Interest of the Child and Genital Mutilation," *Oxford Journal of Legal Studies* 40, no. 3 (2020): 508–32, https://doi.org/10.1093/ojls/gqaa001; Kate Goldie Townsend, "The Child's Right to Genital Integrity," *Philosophy & Social Criticism* 46, no. 7 (2020): 878–98, https://doi.org/10.1177/0191453719854212; Kate Goldie Townsend, "Defending an Inclusive Right to Genital and Bodily Integrity for Children," *International Journal of Impotence Research*, 2022, in press.

[25] Earp, "Against Legalizing Female 'Circumcision' of Minors."

[26] Surya Monro et al., "Intersex: Cultural and Social Perspectives," *Culture, Health & Sexuality* 23, no. 4 (2021): 431–40, https://doi.org/10.1080/13691058.2021.1899529.

[27] Cheryl Chase, "Hermaphrodites with Attitude: Mapping the Emergence of Intersex Political Activism," *GLQ: A Journal of Lesbian and Gay Studies* 4, no. 2 (1998): 189–211, https://doi.org/10.1215/10642684-4-2-189; Alice Dreger, *Intersex in the Age of Ethics* (Hagerstown, MD: University Publishing Group, 1999); Katrina Karkazis, *Fixing Sex: Intersex, Medical Authority, and Lived Experience* (Durham: Duke University Press, 2008); Elizabeth Reis, *Bodies in Doubt: An American History of Intersex* (Baltimore: JHU Press, 2009).

[28] Morgan Carpenter, "Intersex Human Rights, Sexual Orientation, Gender Identity, Sex Characteristics and the Yogyakarta Principles plus 10," *Culture, Health & Sexuality* 23, no. 4 (2021): 516–32, https://doi.org/10.1080/13691058.2020.1781262; Robin Dembroff, "Beyond Binary: Genderqueer as Critical Gender Kind," *Philosophers' Imprint* 20, no. 9 (2020): 1–23.

[29] Vadim M. Shteyler, Jessica A. Clarke, and Eli Y. Adashi, "Failed Assignments — Rethinking Sex Designations on Birth Certificates," *New England Journal of Medicine* 383, no. 25 (2020): 2399–2401, https://doi.org/10.1056/NEJMp2025974.

[30] Elizabeth Barnes, "Gender and Gender Terms," *Noûs* 54, no. 3 (2020): 704–30, https://doi.org/10.1111/nous.12279; Alice H. Eagly and Sabine Sczesny, "Gender Roles in the Future? Theoretical Foundations and Future Research Directions," *Frontiers in Psychology* 10 (2019): online ahead of print, https://doi.org/10.3389/fpsyg.2019.01965; Brian D. Earp, "What Is Gender For?," *The Philosopher* 108, no. 1 (2020): 94–99.

[31] Marie Fox and Michael Thomson, "Foreskin Is a Feminist Issue," *Australian Feminist Studies* 24, no. 60 (2009): 195–210, https://doi.org/10.1080/08164640902852415; Elizabeth Reis, "Intersex Surgeries, Circumcision, and the Making of 'Normal,'" in *Genital Cutting: Protecting Children from Medical, Cultural, and Religious Infringements*, ed. George C. Denniston, Frederick M. Hodges, and Marilyn Fayre Milos (Dordrecht: Springer Netherlands, 2013), 137–47, http://link.springer.com/10.1007/978-94-007-6407-1_10; Edmund M. Horowicz, "Intersex Children: Who Are We Really Treating?," *Medical Law International* 17, no. 3 (2017): 183–218, https://doi.org/10.1177/0968533217726109; Michela Fusaschi, "Gendered Genital Modfifications in Critical Anthropology: From Discourses on FGM/C to New Technologies in the Sex/Gender System," *International Journal of Impotence Research*, 2022, in press.

[32] Sarah M. Creighton, "Surgery for Intersex," *Journal of the Royal Society of Medicine* 94, no. 5 (2001): 218–20; Nancy Ehrenreich and Mark Barr, "Intersex Surgery, Female Genital Cutting, and the Selective Condemnation of Cultural Practices," *Harvard Civil Rights-Civil Liberties Law Review* 40, no. 1 (2005): 71–140, https://pdfs.semanticscholar.org/a986/deba1d02e1035596bfde5befe171eaa95252.pdf; Morgan Carpenter, "The Human Rights of Intersex People: Addressing Harmful Practices and Rhetoric of Change," *Reproductive Health Matters* 24, no. 47 (2016): 74–84, https://doi.org/10.1016/j.rhm.2016.06.003; Elizabeth Reis, "Did Bioethics Matter? A History of Autonomy, Consent, and Intersex Genital Surgery," *Medical Law Review* 27, no. 4 (2019): 658–74, https://doi.org/10.1093/medlaw/fwz007; Renata Ziemińska, "The Epistemic Injustice Expressed in 'Normalizing' Surgery on Children with Intersex Traits," *Diametros* 17, no. 66 (2020): 52–65; Samuel Reis-Dennis and Elizabeth Reis, "The Irrelevance of Data to the Ethics of Intersex Surgery," *Journal of Pediatric Ethics* 1, no. 4 (2021): 162–64.

[33] Milton Diamond and Jameson Garland, "Evidence Regarding Cosmetic and Medically Unnecessary Surgery on Infants," *Journal of Pediatric Urology* 10, no. 1 (2014): 2–6, https://doi.org/10.1016/j.jpurol.2013.10.021.

[34] Anne Fausto-Sterling, "Gender/Sex, Sexual Orientation, and Identity Are in the Body: How Did They Get There?," *Journal of Sex Research* 56, no. 4–5 (2019): 529–55, https://doi.org/10.1080/00224499.2019.1581883; Katharine Jenkins, "Toward an Account of Gender Identity," *Ergo, an Open Access Journal of Philosophy* 5, no. 27 (2018): 713–44, http://dx.doi.org/10.3998/ergo.12405314.0005.027; Talia Mae Bettcher, "Feminist Perspectives on Trans Issues," in *The Stanford Encyclopedia of Philosophy*, ed. Edward N. Zalta, Spring 2014 (Metaphysics Research Lab, Stanford University, 2014), https://plato.stanford.edu/archives/spr2014/entries/feminism-trans/; Robin Dembroff, "Real Talk on the Metaphysics of Gender," *Philosophical Topics* 46, no. 2 (2018): 21–50.

[35] Virginia Hall, "A Transgirl Speaks about Foreskin," *Medium* (blog), May 10, 2017, https://medium.com/@Virginiahallath/a-transgirl-speaks-about-foreskin-53a4f625bb9d.
[36] Nikolaos A. Papadopulos et al., "Quality of Life and Patient Satisfaction Following Male-to-Female Sex Reassignment Surgery," *Journal of Sexual Medicine* 14, no. 5 (2017): 721–30, https://doi.org/10.1016/j.jsxm.2017.01.022; Anthony N. Almazan and Alex S. Keuroghlian, "Association between Gender-Affirming Surgeries and Mental Health Outcomes," *JAMA Surgery* 156, no. 7 (2021): 611–18, https://doi.org/10.1001/jamasurg.2021.0952.
[37] Geolani W. Dy et al., "Reconstructive Management Pearls for the Transgender Patient," *Current Urology Reports* 19, no. 36 (2018): 1–8, https://doi.org/10.1007/s11934-018-0795-y; Dries Opsomer et al., "Creation of Clitoral Hood and Labia Minora in Penile Inversion Vaginoplasty in Circumcised and Uncircumcised Transwomen:," *Plastic and Reconstructive Surgery* 142, no. 5 (2018): 729e–33, https://doi.org/10.1097/PRS.0000000000004926.
[38] Werker, Terng, and Kon, "The Prepuce Free Flap."
[39] Serkan Yldrm et al., "The Preputium: An Overlooked Skin Graft Donor Site," *Annals of Plastic Surgery* 46, no. 6 (2001): 630–34; Mohamed A. Baky Fahmy, "Medical and Surgical Uses of the Prepuce," in *Normal and Abnormal Prepuce* (Cham: Springer International Publishing, 2020), 75–81, https://doi.org/10.1007/978-3-030-37621-5.
[40] Adapted from BCBI, "Medically Unnecessary Genital Cutting and the Rights of the Child: Moving toward Consensus," *The American Journal of Bioethics* 19, no. 10 (2019): 17–28, https://doi.org/10.1080/15265161.2019.1643945; Brian D. Earp, "The Child's Right to Bodily Integrity," in *Ethics and the Contemporary World*, ed. David Edmonds (Abingdon and New York: Routledge, 2019), 217–35.
[41] AAP, "Male Circumcision (Technical Report)," *Pediatrics* 130, no. 3 (2012): e756–85, https://doi.org/10.1542/peds.2012-1990.
[42] Frisch et al., "Cultural Bias in the AAP's 2012 Technical Report and Policy Statement on Male Circumcision."
[43] Robert Darby, "Risks, Benefits, Complications and Harms: Neglected Factors in the Current Debate on Non-Therapeutic Circumcision," *Kennedy Institute of Ethics Journal* 25, no. 1 (2015): 1–34; George C. Denniston, "The Harm of Circumcision," in *Genital Cutting: Protecting Children from Medical, Cultural, and Religious Infringements*, ed. George C. Denniston, Frederick M. Hodges, and Marilyn Fayre Milos (Dordrecht: Springer Netherlands, 2013), 59–68, http://link.springer.com/10.1007/978-94-007-6407-1_4.
[44] Priscilla Alderson, "Children's Consent and the Zone of Parental Discretion," *Clinical Ethics* 12, no. 2 (2017): 55–62, https://doi.org/10.1177/1477750917691887; Rosalind McDougall, Clare Delany, and Lynn Gillam, "The Value of Open Deliberation in Clinical Ethics, and the Role of Parents' Reasons in the Zone of Parental Discretion," *The American Journal of Bioethics* 18, no. 8 (2018): 47–49, https://doi.org/10.1080/15265161.2018.1485773.
[45] For a related argument, see Clare Chambers, *Intact: A Defense of the Unmodified Body* (New York: Penguin, 2022).
[46] For discussion, see Joseph Mazor, "The Child's Interests and the Case for the Permissibility of Male Infant Circumcision," *Journal of Medical Ethics* 39, no. 7 (2013): 421–28, https://doi.org/10.1136/medethics-2013-101318; Eldar Sarajlic, "Can Culture Justify Infant Circumcision?," *Res Publica* 20, no. 4 (2014): 327–43, https://doi.org/10.1007/s11158-014-9254-x; Eldar Sarajlic, "Children, Culture, and Body Modification," *Kennedy Institute of Ethics Journal* 30, no. 2 (2020): 167–90.
[47] Pew Research, "Young Adults around the World Are Less Religious" (Washington, DC: Pew Research Center, June 13, 2018), https://www.pewforum.org/2018/06/13/young-adults-around-the-world-are-less-religious-by-several-measures/.
[48] Kai Möller, "Ritual Male Circumcision and Parental Authority," *Jurisprudence* 8, no. 3 (2017): 461–79, https://doi.org/10.1080/20403313.2017.1339535.
[49] Earp and Darby, "Circumcision, Sexual Experience, and Harm."
[50] Ellen Gruenbaum, *The Female Circumcision Controversy: An Anthropological Perspective* (Philadelphia: University of Pennsylvania Press, 2001).
[51] For further discussion, see Brian D. Earp, "Do the Benefits of Male Circumcision Outweigh the Risks? A Critique of the Proposed CDC Guidelines," *Frontiers in Pediatrics* 3, no. 18 (2015): 1–6, https://doi.org/10.3389/fped.2015.00018; Morten Frisch and Brian D. Earp, "Circumcision of Male Infants and Children as a Public Health Measure in Developed Countries: A Critical Assessment of Recent Evidence," *Glob Pub Health* 13, no. 5 (2018): 626–41, https://doi.org/10.1080/17441692.2016.1184292.
[52] AAP, "Circumcision Policy Statement," *Pediatrics* 130, no. 3 (2012): 585–86, https://doi.org/10.1542/peds.2012-1989.
[53] Frisch et al., "Cultural Bias in the AAP's 2012 Technical Report and Policy Statement on Male Circumcision."
[54] CPS, "Newborn Male Circumcision," *Paediatrics & Child Health* 20, no. 6 (2015): 311–20.
[55] DMA, "Lægeforeningens Politik Vedrørende Omskæring Af Drengebørn Uden Medicinsk Indikation."
[56] For context and citations, see Kirsten Bell, "HIV Prevention: Making Male Circumcision the 'Right' Tool for the Job," *Global Public Health* 10, no. 5–6 (2015): 552–72, https://doi.org/10.1080/17441692.2014.903428. For an

ethical critique of these trials, see Max Fish et al., "A New Tuskegee? Unethical Human Experimentation and Western Neocolonialism in the Mass Circumcision of African Men," *Developing World Bioethics* 21, no. 4 (2021): 211–26.

[57] Maria J Wawer et al., "Circumcision in HIV-Infected Men and Its Effect on HIV Transmission to Female Partners in Rakai, Uganda: A Randomised Controlled Trial," *The Lancet* 374, no. 9685 (2009): 229–37, https://doi.org/10.1016/S0140-6736(09)60998-3.

[58] Marge Berer, "Male Circumcision for HIV Prevention: Perspectives on Gender and Sexuality," *Reproductive Health Matters* 15, no. 29 (2007): 45–48, https://doi.org/10.1016/S0968-8080(07)29304-8; Marge Berer, "Male Circumcision for HIV Prevention: What about Protecting Men's Partners?," *Reproductive Health Matters* 16, no. 32 (2008): 171–75, https://doi.org/10.1016/S0968-8080(08)32418-5; Marge Berer, "Male Circumcision and Risk of HIV in Women," *The Lancet* 374, no. 9700 (2009): 1497–98, https://doi.org/10.1016/S0140-6736(09)61898-5.

[59] First, the baseline prevalence of penile circumcision in the United States is already high, not low, and most HIV in the U.S. is not heterosexually transmitted, but is rather transmitted among men who have sex with men and injecting drug users, as in many other Global North contexts. The overall prevalence of HIV is also much lower. Jennifer A. Bossio, Caroline F. Pukall, and Stephen S. Steele, "A Review of the Current State of the Male Circumcision Literature," *Journal of Sexual Medicine* 11, no. 12 (2014): 2847–64, https://doi.org/10.1111/jsm.12703; Jennifer A. Bossio, Caroline F. Pukall, and Stephen Steele, "Response to: 'The Literature Supports Policies Promoting Neonatal Male Circumcision in N. America,'" *Journal of Sexual Medicine* 12, no. 5 (2015): 1306–7, https://doi.org/10.1111/jsm.12852; Madhur Nayan et al., "Circumcision and Risk of HIV among Males from Ontario, Canada," *Journal of Urology*, no. 0 (2021): online ahead of print, https://doi.org/10.1097/JU.0000000000002234; Morten Frisch and Jacob Simonsen, "Non-Therapeutic Male Circumcision in Infancy or Childhood and Risk of Human Immunodeficiency Virus and Other Sexually Transmitted Infections: National Cohort Study in Denmark," *European Journal of Epidemiology*, 2021, in press.

[60] Matthew Deacon and Gordon Muir, "What Is the Medical Evidence on Non-Therapeutic Child Circumcision?," *International Journal of Impotence Research*, 2022, online ahead of print, https://doi.org/10.1038/s41443-021-00502-y.

[61] J. Steven Svoboda, "Nontherapeutic Circumcision of Minors as an Ethically Problematic Form of Iatrogenic Injury," *AMA Journal of Ethics* 19, no. 8 (2017): 815–24, https://doi.org/10.1001/journalofethics.2017.19.8.msoc2-1708.

[62] Brian D. Earp, Lauren Sardi, and William Jellison, "False Beliefs Predict Increased Circumcision Satisfaction in a Sample of US American Men," *Culture, Health & Sexuality* 20, no. 8 (2018): 945–59, https://doi.org/10.1080/13691058.2017.1400104.

[63] Ronald Goldman, "The Psychological Impact of Circumcision," *BJU International* 83, no. S1 (1999): 93–102, https://doi.org/10.1046/j.1464-410x.1999.0830s1093.x.

[64] BCBI, "Medically Unnecessary Genital Cutting and the Rights of the Child," p. 18.

[65] Robert S. Van Howe, "Effect of Confounding in the Association between Circumcision Status and Urinary Tract Infection," *The Journal of Infection* 51, no. 1 (2005): 59–68, https://doi.org/10.1016/j.jinf.2004.07.003.

[66] AAP, "Male Circumcision (Technical Report)."

[67] NHS, "Urinary Tract Infection (UTI) in Children" (National Health Service, 2020), https://www.nhsinform.scot/illnesses-and-conditions/kidneys-bladder-and-prostate/urinary-tract-infection-uti-in-children; Steven L. Chang and Linda D. Shortliffe, "Pediatric Urinary Tract Infections," *Pediatric Clinics* 53, no. 3 (2006): 379–400, https://doi.org/10.1016/j.pcl.2006.02.011.

[68] Aaron J. Krill, Lane S. Palmer, and Jeffrey S. Palmer, "Complications of Circumcision," *The Scientific World JOURNAL* 11 (2011): 2458–68, https://doi.org/10.1100/2011/373829.

[69] Joao L. Pippi Salle et al., "Glans Amputation during Routine Neonatal Circumcision: Mechanism of Injury and Strategy for Prevention," *Journal of Pediatric Urology* 9, no. 6, Part A (2013): 763–68, https://doi.org/10.1016/j.jpurol.2012.09.012.

[70] Scott Campbell, "Risk and the Subjectivity of Preference," *Journal of Risk Research* 9, no. 3 (2006): 225–42, https://doi.org/10.1080/13669870600603147.

[71] John M. Hutson, "Circumcision: A Surgeon's Perspective," *Journal of Medical Ethics* 30, no. 3 (June 1, 2004): 238–40, https://doi.org/10.1136/jme.2002.001313.

[72] Rebecca Dekker and Anna Bertone, "Evidence and Ethics on: Circumcision," *Evidence Based Birth* Signature Article (2019): 1–44.

[73] Mohamed A. Baky Fahmy, "Complications of Male Circumcision (MC)," in *Congenital Anomalies of the Penis* (Cham: Springer, 2017), 227–35, https://doi.org/10.1007/978-3-319-43310-3_35.

[74] Robert Darby, "To Avoid Circumcision Complications, Avoid Circumcision," *Canadian Urological Association Journal* 8, no. 7–8 (2014): 231, https://doi.org/10.5489/cuaj.1709.

[75] Aaron AR Tobian and Ronald H. Gray, "The Medical Benefits of Male Circumcision," *JAMA* 306, no. 13 (2011): 1479–80. Thank you to one of the editors for raising this point.

[76] John R. Holman and Keith A. Stuessi, "Adult Circumcision," *American Family Physician* 59, no. 6 (1999): 1514–18.
[77] Helen A. Weiss et al., "Complications of Circumcision in Male Neonates, Infants and Children: A Systematic Review," *BMC Urology* 10, no. 2 (2010): 1–13, https://doi.org/10.1186/1471-2490-10-2.
[78] Tobian and Gray, "The Medical Benefits of Male Circumcision."
[79] Brian J. Morris et al., "A 'snip' in Time: What Is the Best Age to Circumcise?," *BMC Pediatrics* 12, no. 20 (2012): 1–15, p. 4.
[80] The claim is contestable, in part, because there are no high-quality studies that have directly compared routine infant circumcision with later, consensual adult circumcision, controlling for all relevant confounds, such as whether the procedure is therapeutic or non-therapeutic. Rather the comparisons are typically between different studies, or using different databases, reflecting circumcisions done at different ages in different settings by different practitioners, etc. See Myers and Earp, "What Is the Best Age to Circumcise?"
[81] Campbell, "Risk and the Subjectivity of Preference."
[82] Clare Chambers, "Reasonable Disagreement and the Neutralist Dilemma: Abortion and Circumcision in Matthew Kramer's 'Liberalism with Excellence,'" *The American Journal of Jurisprudence* 63, no. 1 (2018): 9–32, https://doi.org/10.1093/ajj/auy006; B. Lyons, "Male Infant Circumcision as a 'HIV Vaccine,'" *Public Health Ethics* 6, no. 1 (2013): 90–103, https://doi.org/10.1093/phe/phs039.
[83] Stephen R Munzer, "Examining Nontherapeutic Circumcision," *Health Matrix* 28, no. 1 (2018): 1–77, https://scholarlycommons.law.case.edu/healthmatrix/vol28/iss1/5/.
[84] BCBI, "Medically Unnecessary Genital Cutting and the Rights of the Child," p. 18.
[85] G. J. Boyle et al., "Circumcision of Healthy Boys: Criminal Assault?," *Journal of Law and Medicine* 7, no. 2 (2000): 301–10; Reinhard Merkel and Holm Putzke, "After Cologne: Male Circumcision and the Law. Parental Right, Religious Liberty or Criminal Assault?," *Journal of Medical Ethics* 39, no. 7 (2013): 444–49, https://doi.org/10.1136/medethics-2012-101284; Brian D. Earp, "Why Was the U.S. Ban on Female Genital Mutilation Ruled Unconstitutional, and What Does This Have to Do with Male Circumcision?," *Ethics, Medicine and Public Health* 15, no. 100533 (2020): 1–13, https://doi.org/10.1016/j.jemep.2020.100533; Christopher Price, "Male Circumcision: An Ethical and Legal Affront," *Bulletin of Medical Ethics* 128, no. 5 (1997): 13–19; Margaret Sommerville, *The Ethical Canary: Science, Society, and the Human Spirit* (Montreal: McGill-Queen's University Press, 2004); J. Steven Svoboda, Peter W. Adler, and Robert S. Van Howe, "Circumcision Is Unethical and Unlawful," *The Journal of Law, Medicine & Ethics* 44, no. 2 (2016): 263–82, https://doi.org/10.1177/1073110516654120; J. Steven Svoboda, Peter W. Adler, and Robert S. Van Howe, "Is Circumcision Unethical and Unlawful? A Response to Morris et Al.," *The Journal of Medical Law and Ethics* 7, no. 1 (2019): 72–92.
[86] David Archard, "The Wrong of Rape," *The Philosophical Quarterly* 57, no. 228 (2007): 374–93, https://doi.org/10.1111/j.1467-9213.2007.492.x; Brian D. Earp and Joshua Yuter, "Is Circumcision Wrong?," *Letter*, September 3, 2019, https://letter.wiki//conversation/127.
[87] Earp and Yuter, "Is Circumcision Wrong?"
[88] Marie Fox and Michael Thomson, "Bodily Integrity, Embodiment, and the Regulation of Parental Choice," *Journal of Law and Society* 44, no. 4 (2017): 501–31, https://doi.org/10.1111/jols.12056; Jonathan Herring and Jesse Wall, "The Nature and Significance of the Right to Bodily Integrity," *The Cambridge Law Journal* 76, no. 3 (2017): 566–88, https://doi.org/10.1017/S0008197317000605.
[89] A possible exception is the case of Jehovah's Witnesses (JW), many of whom, based on their metaphysical beliefs, would prefer to die than to undergo certain medically necessary procedures, most notably blood transfusions. However, in the case of minor children raised within JW families, if there is even a slim chance (and there surely is) that they might one day leave their religious community or abandon the highly unusual belief system of their parents—or even just the specific belief that it is better to die than to receive a blood transfusion—then it is wrong to withhold the transfusion, assuming no available bloodless options. It is precisely the fact that we cannot be sufficiently certain of the child's future metaphysical beliefs—and hence what they would, in fact, consent to upon becoming autonomous—that entitles us to give the necessary infusion, thereby preserving and protecting their future bodily autonomy (that is, leaving the decision about life or death to them when they are mature enough to face it given their adult values and commitments). For a further interesting analysis of the JW case, see Niklas Juth and Niels Lynøe, "Are There Morally Relevant Differences between Hymen Restoration and Bloodless Treatment for Jehovah's Witnesses?," *BMC Medical Ethics* 15, no. 89 (2014): 1–7, https://doi.org/10.1186/1472-6939-15-89.
[90] This argument is developed in: Earp, "The Child's Right to Bodily Integrity." For a critical discussion, see Chambers, *Intact: A Defense of the Unmodified Body*.
[91] Chambers, "Reasonable Disagreement and the Neutralist Dilemma"; Möller, "Ritual Male Circumcision and Parental Authority"; Sarajlic, "Children, Culture, and Body Modification."

[92] For recent discussions of the need for gender-neutral definitions of rape and sexual assault, see David M. Bierie and James C. Davis-Siegel, "Measurement Matters: Comparing Old and New Definitions of Rape in Federal Statistical Reporting," *Sexual Abuse* 27, no. 5 (2015): 443–59, https://doi.org/10.1177/1079063214521470; Natasha McKeever, "Can a Woman Rape a Man and Why Does It Matter?," *Criminal Law and Philosophy* 13, no. 4 (2019): 599–619, https://doi.org/10.1007/s11572-018-9485-6; Scott M. Walfield, "'Men Cannot Be Raped': Correlates of Male Rape Myth Acceptance," *Journal of Interpersonal Violence* 36, no. 13–14 (2021): 6391–6417, https://doi.org/10.1177/0886260518817777.

[93] Namely those in which such touching is required to protect and preserve the child's future bodily integrity and sexual autonomy (for example, necessary medical exams for diagnosis or treatment, or a parent helping a young child wash their genitalia when the child cannot yet do so on their own).

[94] Brian D. Earp, "Zero Tolerance for Genital Mutilation: A Review of Moral Justifications," *Current Sexual Health Reports* 12, no. 1 (2020): 276–88, https://doi.org/10.1007/s11930-020-00286-0.

[95] George C. Denniston and Marilyn Fayre Milos, eds., *Sexual Mutilations: A Human Tragedy* (Boston, MA: Springer US, 1997), https://doi.org/10.1007/978-1-4757-2679-4.

[96] Markus Bauer, Daniela Truffer, and Daniela Crocetti, "Intersex Human Rights," *The International Journal of Human Rights* 24, no. 6 (2020): 724–49, https://doi.org/10.1080/13642987.2019.1671354.

[97] Waldeck, "Social Norm Theory and Male Circumcision."

[98] Fox and Thomson, "Foreskin Is a Feminist Issue"; see also Ralph Sandland, "The Construction of Gender and Sexuality in the Approach of Key International Law Actors to the Circumcision of Children," *Human Rights Law Review* 19, no. 4 (2019): 617–47, https://doi.org/10.1093/hrlr/ngz030.

[99] Tim Hammond and Adrienne Carmack, "Long-Term Adverse Outcomes from Neonatal Circumcision Reported in a Survey of 1,008 Men: An Overview of Health and Human Rights Implications," *The International Journal of Human Rights* 21, no. 2 (February 12, 2017): 189–218, https://doi.org/10.1080/13642987.2016.1260007.

[100] Richard A. Shweder, "'What about Female Genital Mutilation?' And Why Understanding Culture Matters in the First Place," in *Engaging Cultural Differences: The Multicultural Challenge in Liberal Democracies*, ed. Richard A. Shweder, Martha Minow, and Hazel R. Markus (New York: Russell Sage Foundation Press, 2002), 216–51.

[101] Joann M. Ross, "Making Marital Rape Visible: A History of American Legal and Social Movements Criminalizing Rape in Marriage" (Doctoral Dissertation, United States - Nebraska, The University of Nebraska - Lincoln, 2015), https://www.proquest.com/docview/1749782227/abstract/CDA979960A9A4506PQ/1.

[102] Goldman, "The Psychological Impact of Circumcision"; Sarah E. Waldeck, "Using Male Circumcision to Understand Social Norms as Multipliers," *University of Cincinnati Law Review* 72 (2003): 455–526.

[103] I explain this position at great length in Earp, "Against Legalizing Female 'circumcision' of Minors."

[104] See Dena S. Davis, "Male and Female Genital Alteration: A Collision Course with the Law," *Health Matrix* 11, no. 1 (2001): 487–570.